Ethical principles and withdrawal of treatment

Four principles

- Beneficence, non-maleficence, autonomy, justice

Withdrawal of treatment

- Equivalent to withholding ethically; decision by consensus
- Family meeting, document, palliative care

Practical aspects

- Symptom control (opioids, sedatives), stop non-comfort treatments, family presence

Key point: Withdrawal is a change of goals, not of care.

Mental capacity and refusal of treatment

Capacity

- Understand, retain, weigh information, communicate decision
- Decision-specific; presume capacity

Management

- Respect competent refusal (Jehovah's Witness): cell salvage, EPO, iron, TXA

Unconscious patient

- Advance directive valid → respect it; otherwise best interests

Key point: A capable adult may refuse life-saving treatment.

ICU admission, DNACPR and futility

Admission factors

- Reversibility, patient wishes, comorbidity, frailty, likely benefit

DNACPR

- Decision not to attempt CPR; discussed with patient/family, reviewed

Futility

- Treatment with no reasonable chance of benefit; value-laden concept

Key point: DNACPR does not mean do not treat.

Breaking bad news and family conflict

Structured approach (SPIKES)

- Setting, perception, invitation, knowledge, emotions, strategy/summary

Supporting families

- Regular meetings, consistent message, diaries, psychology

Conflict

- Listen, second opinion, mediation, ethics committee

Key point: Communication prevents most ICU conflicts.

Guidelines (no 2026 note on this topic)

Organ donation

Ethical principles

- Dead donor rule, consent, separation of donation and treatment teams

DBD vs DCD

- DBD: brain death; DCD: circulatory death after withdrawal (5 min stand-off)

Family conflict

- Specialist nurse, respect wishes, time

Key point: Separate end-of-life decisions from donation.

Guidelines (no 2026 note on this topic)

Pandemic triage

Frameworks

- Utilitarian (most lives saved), egalitarian, fairness, transparency

Criteria

- SOFA, frailty, likely benefit; not age or disability alone

Patients not allocated

- Palliative care, reassessment, communication

Key point: Triage must be transparent and consistent.

Guidelines (no 2026 note on this topic)

Medication error and disclosure

Disclosure

- Duty of candour: apologise, explain, support

Investigation

- Root cause analysis, no blame, learning

System measures

- Smart pumps, double checks, standard concentrations, electronic prescribing

Key point: Errors are system failures: fix the system.

Research ethics in ICU

Valid consent

- Informed, voluntary, capacity

Incapacitated patients

- Deferred consent, proxy consent, ethics committee approval

Issues

- Vulnerability, equipoise, data protection

Key point: Research needs equipoise and protection of the vulnerable.

Guidelines (no 2026 note on this topic)